Aster
RV HOSPITAL
We'll Treat You Well

# DISCHARGE SUMMARY

| | | | |
|---|---|---|---|
| UHID: | ABH0020000020791 | Patient No: | IR20001899 |
| Patient Name: | Mr.BABU P A | Age ( Gender ): | 58Y ( Male ) |
| Admission Date: | 06/04/2020 15:04 | DischargeOn | 13/05/2020 |
| Bed No: | 506E(FIFTH FLOOR RV) | Discharge Type: | DAMA |
| Sponsor: | SELF | Primary Physician: | NEUROSURGERY TEAM |
| Clinical Dept: | NEURO SURGERY | Phone: | 7356869754 |
| Line of Treatment: | SURGERY | Date of Operation: | |
| Address: | M B CASTLE APARTMENT FLAT NO 402. KALENA AGRAHARA, GOTTIGERE. BANGALORE. KARNATAKA. INDIA. 560076. | | |

**REASON FOR ADMISSION**

Acute hemorrhagic stroke (left capsillo ganglionic bleed)

**DISCHARGE DIAGNOSIS**

Left capsuloganglionic bleed secondary to ruptured left parietal AVM

**HISTORY/ COMORBIDITIES**

History of sudden onset of vomiting followed by altered sensorium since 1.30pm on 02-04-2020 while he was at ATM
Patient is in altered sensorium since then, was initially taken to NIMHANS and subsequently was treated at a private hospital (Sai Ambika hospital)
He was on ventillator till previous night(03-04-2020) and now on endotracheal tube with oxygen suppliment
Patient attenders have noted right side weakness and aphasia since then
He is a known smoker and alcoholic since 30 years

**ALLERGIES**

No known allergies

**CLINICAL EXAMINATION:**

GCS- E1M3Vt
Pupils bilateral 2mm sluggish
Moving left upper limb only, right hemiplegia
Febrile-101
BP- 170/110 mmHg

**INVESTIGATIONS DONE**

MRI brain(04-04-2020) at Ragav diagnostic- left capsuloganglionic bleed with abnormal conglomerate of vessels in the left parietal region suggestive of ?AVM, mass effect present, midline shift present.

CT brain(07-04-2020)-
Diffuse intraparenchymal hemorrhage noted on left side involving temporal ,frontoparietal region, Left basal ganglia with extension into left lateral ventricle, 3rd ventricle, 4th ventricle and rightLateral ventricle.
Perilesional gross edema noted.
Diffuse subarachnoid hemorrhage noted in bilateral cerebral hemisphere.
Calcifications noted in left parietal area
Mid line shift to right side of about 11.4 mm.
Cerebral atrophy noted.
Rest of the brain parenchyma shows normal attenuation

**PROCEDURE DONE/OPERATIVE NOTES**

Left temporal craniotomy and evacuation of hematoma with tracheostomy under GA on 07-04-2020.

Findings - Dura tense and bulging, abnormal dilated veins seen on the posterior temporal



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cortex and sroumnd the sylvian fissure, cortisectomy done on area of middle temporal gyrus which had no abnormal vessels, hematoma evacuated completly, hemostasis achieved, brain lax and pulsatile.

**COURSE IN THE HOSPITAL**

This 58 year old gentleman presented with above mentioned history and findings. After overnight observation and once fever settled (patient already on injection piptaz with endotracheal tube) CT head scan was done. Size of the haematoma and midline shift was significant. In view of poor GCS and large haematoma, it was decided to evacuate the haematoma. Patient's attendants (daughter and son-in-law) were explained about the need of surgery to evacuate the blood clot and reduce the risk to his life. They were also explained about right sided hemiplegia(weakness)and speech disturbance may not recover even after surgery. They were also explained that there is a underline AVM (vascular malformation) mainly on the left side of the brain which may need definitive proceedure to diagnose and to treat subsequently once he recovers from present crisis. They were amply explained about the need for artifitial ventilation in the post operative period and prolonged stay in ICU and in hospital and extensive neuro-rehabilitation program. Financial implications of the surgical proceedure, prolonged stay and further care were also explained. With their consent, patient underwent above mentioned procedure including tracheostomy. Post operatively patient was ventillated and was gradually weaned off the ventillator on 11-04-2020. He was administered appropriate antibiotics and antiedema measures. He showed gradual improvement in his sensorium to a level of E2M5Vt. He required oxygen suppliment through tracheostomy. He was shifted to wards on 15-04-2020. In the ward he has been showing gradual improvement in his sensorium to the state of GCS E4M6Vt with persistent right hemiplegia, reduced movement of left lower limb, patient is able to emotionally comprehend what is being said and responds to simple verbal commands. Patient has remained afebrile, maintaining saturation at room air, with no pressure sores. Patient had recurrent episodes of loose stools, not subsiding to medical management hence gastroenterologist opinion was sought and sigmoidoscopy was done on 07-05-2020 to rule out colitis currently loose stools has subsided with course of antibiotics. He is fed by ryles tube and oral feeds. Patient family has been made aware that he needs proper care of tracheostomy and neuro rehabilitation. Patient needs to be taken either to a nursing home or neurohabilitation centre for proper care of tracheostomy and rehabilitation, which has been explained to the family. They have also been made aware that left sided AVM needs evaluation and further treatment in due course.

**MEDICATIONS ADMINISTERED**

Supportive medications, antiedema, antiepileptics, amantadine, analgesics, perioperative antibiotics

**CONDITION AT DISCHARGE**

GCS-E4M6Vt
Pupils equal and reactive
Right hemiplegia
Wound healthy
RT in situ
Tracheostomy in situ
Catheter in situ
Left lower limb minimal movement

**ADVICE ON DISCHARGE**

1) Tab Metronidozole 400mg 1-1-1 x till 21-05-2020
2) Cap VSL #3 1-0-0 x till 21-05-2020
3) Levipil 500 mg 1-0-1 to continue
4) Tab Metformin 500mg 1-0-1 x continue



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5) Tab Amaryl 1mg 1-0-0 x continue
6) Daily bath and hygiene
7) Chest and limb physiotherapy
8) Air bed
9) Frequent position change
10) Back, bowel and bladder care
11) Tracheostomy care/catheter care/ RT care

**DIET ADVICE** Semisolid food-orally with RT feed

**FOLLOW UP DATE AND REVIEW** Follow up after 1week with Neurosurgery team with prior appointment.

**WHEN TO OBTAIN URGENT CARE** Fever, seizure, vomitting, fresh neurological deficits.

**CONSULTANT**

Dr. B A CHANDRAMOULI
M.Ch Neurosurgery(former Professor and HOD Neurosurgery NIMHANS)
LEAD CONSULTANT - NEUROSURGERY

Dr. SUDHIR S PAI
M.S, M.Ch Neurosurgery(AIIMS-New Delhi)
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Dr. DHANANJAYA I BHAT
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SENIOR CONSULTANT - NEUROSURGERY

Dr. SANTHOSH KUMAR S A
M.S, M.Ch Neurosurgery(NIMHANS), DNB
SENIOR SPECIALIST - NEUROSURGERY

For SIGNATURE OF THE CONSULTANT



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